IRRITABLE BOWEL SYNDROME

----- Active space -----

Functional bowel disorder : Reaction of GI system to stress.

ASSOCIATIONS

- Lactose intolerance.
- Celiac disease.
- Fibromyalgia.
- Chronic fatigue syndrome.
- Generalized anxiety disorder.

Clinical Presentations and Pathophysiology

00:01:40

CLINICAL PRESENTATIONS

1. Gender : Female > male.
2. Age : < 45 years.
3. Pain abdomen :
   - most Important.
   - Rome IV criteria : Recurrent abdominal pain, minimum 1 episode/week for ≥3 months, associated with ≥2 of :
     a. Change in stool frequency.
     b. Change in stool form (Appearance).
     c. Related to defecation.
   - Site : Crampy lower abdomen.
   - Frequency : Episodic (1/week).
   - Aggravating factors : Stress, eating.
   - Relieving factors : Defecation.
4. Altered bowel habits :
   - most consistent symptom.
   - Types :
     - D+ : Diarrhoea predominant (Small volume, associated with mucus).
     - C+ : Constipation predominant.
     - m+ (m/C) : mixed (Prolonged constipation interrupted by episodic diarrhoea).
5. Abdomen distension, belching.
6. Dyspepsia (Heart burn).
7. Nausea and vomiting.

Note :

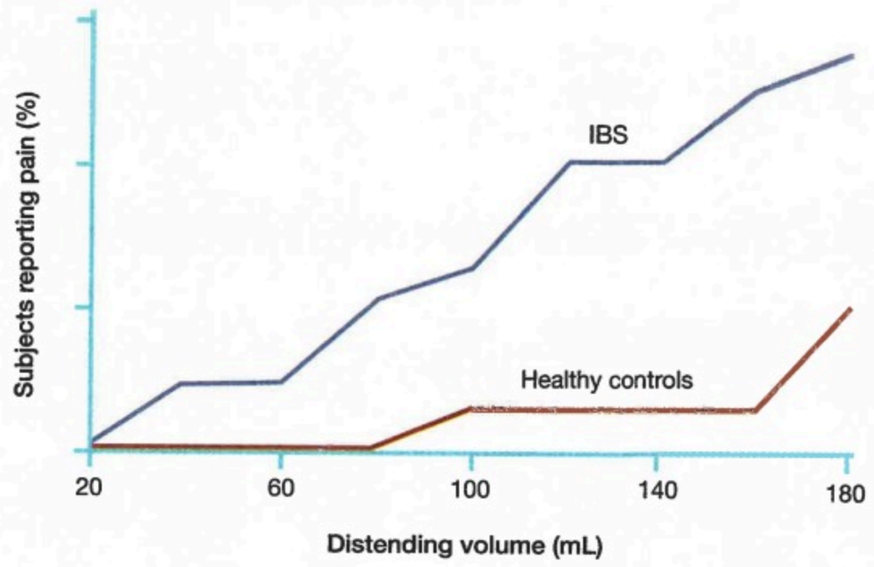
Features against diagnosis of IBS :

- Nocturnal diarrhoea.
- Steatorrhea.
- Blood in stools.
- Fever.
- Weight loss, anemia, malabsorption.
- Increased ESR.

----- Active space -----

PATHOPHYSIOLOGY

1. Theory of visceral hypersensitivity : Minimal rectal distension → Stimulates pain receptors (Enhanced sensitivity).
2. Increased colonic motor potentials.
3. Central activation of cingulate cortex.
4. Increase in serotonin containing enterochromaffin cells.
5. Vanilloid channels (TRP-VI).



Rectal hypersensitivity in IBS

Management

00:11:02

Diet and lifestyle modifications

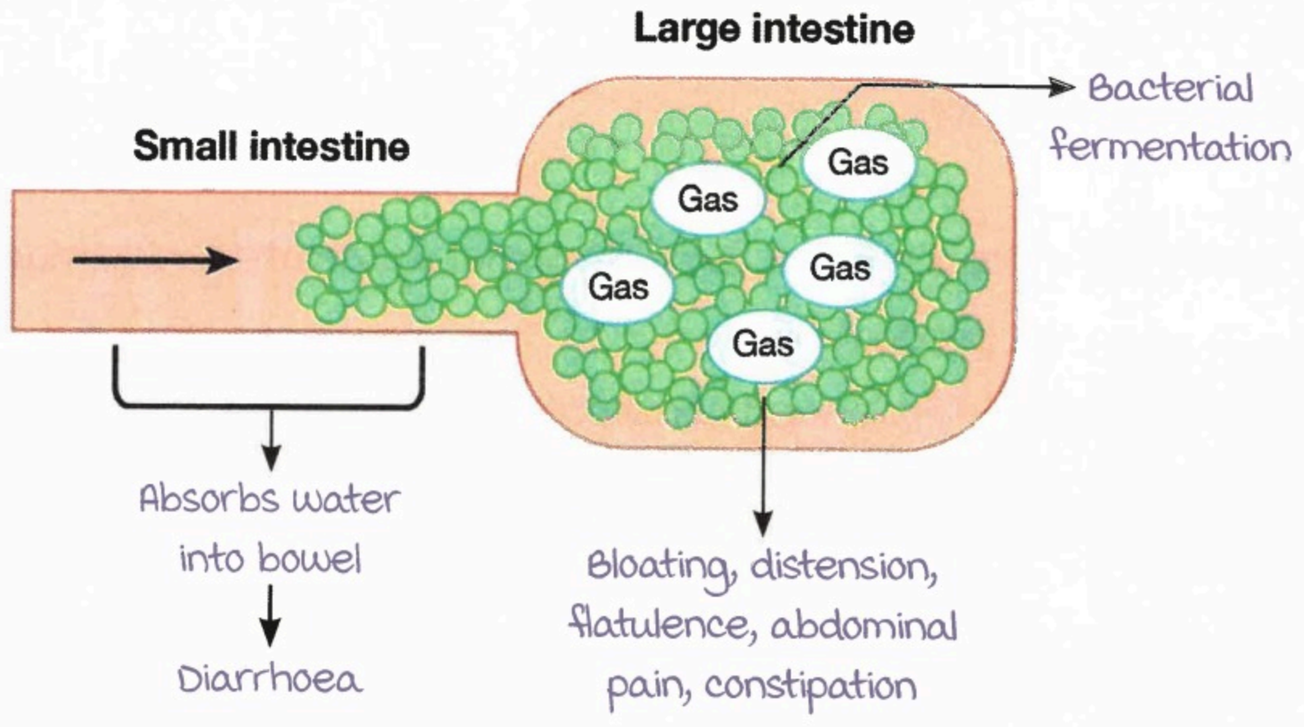
- Improvement (70% cases)
- No improvement (30% cases) → medical management

Walking : 6000-8000 steps.

Diet : Reduce FODMAP.

FODMAPs:

- Fermentable oligo, di, monosaccharides and polyols.
- Sources: Apple, mango, wheat, honey, legume, cauliflower etc.



medical management :

----- Active space -----

Based on type of IBS

D+ :

- Loperamide.
- Antispasmodics : 30 mins before food.
- Antidepressants.
- Obsolete drugs :

Drugs	Action	Side effects
Alosetron	5HT3 antagonist	Ischemic colitis
Eluxadoline	μ, κ-agonist, δ-antagonist	Pancreatitis

C+ :

- Stool bulking agents :
  - Psyllium husk.
  - PEG (Polyethylene glycol) laxatives.
- Lubiprostone (Bicyclic fatty acid → Cl^- channel activator).
- Linaclotide (Guanyl cyclase agonist)
- Tenapanor (Na^+-H^+ exchange blocker)

Note :

- Effectiveness of antibiotics, probiotics, prebiotics not determined.
- Tegaserod is not used d/t cardiac toxicity.